
CLINICAL ENCOUNTER — SOAP NOTE

Date: July 21, 2026

Patient: Brooke Atwood

CC/Reason: Follow-up for hormone therapy adjustment and joint pain management

SUBJECTIVE

HPI:

Brooke Atwood, a 51-year-old female, returns for a follow-up visit focused on managing her hormone therapy and addressing joint pain. She reports experiencing a prolonged menstrual period lasting two weeks, accompanied by constant cramps, which she attributes to recent hormone therapy adjustments. Brooke is currently taking progesterone, two capsules at night, and Estrogel daily. She describes feeling tired and notes persistent joint aches, particularly suspecting tennis elbow. She is considering an orthopedic evaluation to explore options like physical therapy or injections. Brooke expresses interest in increasing her progesterone to 300 mg nightly to better manage her symptoms and is open to compounding testosterone and estrogen into a transdermal gel for ease of use and preference over injections. She has refused testosterone injections, preferring the transdermal route. Brooke is also planning to complete a Cologuard test for GI screening.

Current Medications:

- Progesterone 200 mg oral nightly
- Estrogel transdermal daily

Medical History:

- Allergies: Not reported at this visit
- Past Medical Hx: Elevated cholesterol
- Past Surgical Hx: Not reported at this visit
- Social Hx: Occasional recreational smoking when drinking
- Family Hx: Paternal grandfather with heart attack

ROS

Constitutional: Feels tired; no fever or weight changes reported.

HEENT: Not addressed at this visit.

Cardiovascular: Not addressed at this visit.

Respiratory: Not addressed at this visit.

Gastrointestinal: Not addressed at this visit.

Genitourinary: Reports prolonged menstrual bleeding and cramps.

Musculoskeletal: Joint aches, particularly in elbows.

Skin: Not addressed at this visit.

Neurological: Not addressed at this visit.

Psychiatric: Not addressed at this visit.

Endocrine: Hormonal therapy effects discussed.

Hematologic/Lymphatic: Not addressed at this visit.

Allergic/Immunologic: Not addressed at this visit.

OBJECTIVE

Vitals: Not obtained at this encounter

Physical Exam: Physical examination not performed at this encounter.

ASSESSMENT/PLAN

Brooke Atwood presents with symptoms indicative of hormonal imbalance, characterized by prolonged menstrual bleeding, cramps, fatigue, and joint aches. The therapeutic plan focuses on optimizing hormone therapy to alleviate these symptoms and improve her quality of life.

1. Post-Menopausal Disorder with Symptom Complex (F52.0, R53.83, E34.9)

Brooke's symptoms of prolonged menstrual bleeding and cramps are consistent with hormonal imbalance. Current progesterone dose is insufficient, necessitating an increase to 300 mg nightly to better control bleeding and balance hormone therapy. Discussed the benefits of compounding testosterone and estrogen into a transdermal gel for ease of use and patient preference.

Plan:

Increase progesterone to 300 mg oral nightly. Prescribe compounded testosterone and estrogen transdermal gel daily. Monitor symptom response and adjust therapy as needed. Follow up in 6-8 weeks to assess hormone therapy response and perform labs.

2. Joint Pain (M25.5)

Joint aches, particularly in the elbows, are reported and may benefit from orthopedic evaluation. Discussed options for physical therapy or injections if needed.

Plan:

Schedule orthopedic appointment for evaluation. Patient to inform if referral is needed. Follow up as needed based on orthopedic recommendations.

CARE PLAN

- Increase progesterone to 300 mg by mouth at bedtime to help control bleeding and balance hormone therapy.
- Start compounded testosterone and estrogen transdermal gel daily for hormone therapy.
- Schedule an orthopedic appointment for evaluation of joint pain.
- Complete Cologuard test for GI screening as planned.

- Monitor for any changes in symptoms, particularly joint pain and hormonal effects.
- Follow up in 6-8 weeks to assess hormone therapy response and perform labs.

FOLLOW-UP

Follow up in 6-8 weeks to assess hormone therapy response and perform labs.

A handwritten signature in black ink, appearing to be 'Morgan Pitts', written in a cursive style.

Electronically Signed

Signed by: FNP-C Morgan Pitts (NPI: 1407623762)

Signed on: July 21, 2026 at 11:44 AM

This document has been electronically signed and constitutes a legally valid clinical record.